

SAINT AUGUSTA REGIONAL MEDICAL CENTER

Department of Internal Medicine

PROGRESS NOTE - HOSPITAL DAY 3

MRN: SA-2210984

Age: 74 Sex: Female

Date of Service: 02/09/2026

Attending: Priya Raghunathan, MD NPI: 1487302956

SUBJECTIVE:

Patient admitted three days ago with progressive dyspnea and lower extremity swelling. She reports her breathing is somewhat improved this morning but she remains short of breath with minimal exertion. She denies chest pain. She denies fever.

PAST MEDICAL HISTORY:

Congestive heart failure. Chronic obstructive pulmonary disease. Chronic kidney disease stage 3. Hypertension.

OBJECTIVE:

BP 118/64 HR 92 RR 22 Temp 98.6 F SpO2 91% on 2 L nasal cannula

General: elderly female, mild respiratory distress.

Cardiovascular: JVD to the angle of the jaw. 2+ pitting pedal edema.

Respiratory: diffuse expiratory wheezing with prolonged expiratory phase.

Extremities: warm, no calf tenderness.

LABORATORY:

Hemoglobin 8.9 g/dL, down from 10.2 on admission. MCV 78. Ferritin 14.

Creatinine 1.9. eGFR 32. BNP 1840. Potassium 3.2.

Echocardiogram 02/07/2026: ejection fraction of 35 percent with global hypokinesis.

Reviewed the chest x-ray from admission and discussed with cardiology.

ASSESSMENT AND PLAN:

1. Congestive heart failure - volume overloaded on exam with an EF of 35 percent. Continue furosemide 40 mg IV twice daily. Strict intake and output. Daily weights.
2. Chronic obstructive pulmonary disease - increased sputum production and wheezing since admission, consistent with an acute exacerbation. Continue nebulizer treatment every 6 hours and prednisone taper.
3. Chronic kidney disease stage 3 - creatinine up from baseline 1.5 in the setting of diuresis. eGFR 32. Monitor closely.
4. Anemia - hemoglobin has fallen to 8.9. Ferritin is low at 14. No overt bleeding. Will obtain iron studies and guaiac stools.

5. Hypokalemia - potassium 3.2. Replace with 40 mEq oral potassium chloride.

Chest x-ray 2 views ordered for this morning to reassess.

Electrocardiogram routine 12-lead with interpretation and report performed, showing sinus tachycardia without acute ischemic change.

Continue current level of care. Anticipate two to three more hospital days.